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September 27, 2016

Division of Dockets Management (HFA-305)
Food and Drug Administration
5630 Fishers Lane, Rm. 1061
Rockville, MD 20852

Re: Docket No. FDA-2015-D-3719 for "Draft Guidances Relating to the Regulation of Human Cells, Tissues, and Cellular and Tissue-Based Products; Rescheduling of Public Hearing; Request for Comments"

Dear Sir or Madam:

Parenteau BioConsultants, LLC is pleased to submit the following comments on the Draft Guidances Relating to the Regulation of Human Cells, Tissues, and Cell and Tissue-Based Products. These comments were presented by Janet Hardin-Young, a co-founder of Parenteau BioConsultants, during the Part 15 Hearing held by FDA on September 12, 2016 to discuss these guidance documents. Parenteau BioConsultants provides scientific and regulatory consulting services with a special focus on regenerative medicine, tissue engineering and cell-based therapies. We would like to take this opportunity to address certain important issues raised by the draft guidance documents under discussion which will, potentially, provide much-needed regulatory clarity in a space that has not previously received sufficient attention.

In particular these comments will address the concept of intended use and its application within these guidance documents. As a threshold matter, the purpose of Agency guidance is to clarify existing regulation, and FDA cannot and should not introduce new regulations via guidance. Despite objections to the various ways the guidances incorporate the concept of "intended use," it is, of course, not new. The regulatory status of virtually every product under FDA's jurisdiction turns on the use for which its distributor intends it. In the context of HCT/Ps specifically, the idea that the degree of regulation to which a tissue is subject would turn on its intended use has always been a bedrock principle of the risk-based approach that underpins Part 1271.

Section 1271.10 incorporates the concept of “intended use” most notably in the requirement that Section 361 HCT/Ps must be intended for homologous use. When the regulatory scheme was conceived in 1997, the rationale for this requirement was that homologous-use products can reasonably be exempted from premarket review because a tissue’s behavior for a homologous use is readily predictable. By contrast, products not intended for homologous use require premarket review because clinical trials are necessary to establish the behavior of cells and tissues for such uses. Nevertheless, today the market is crowded with products for which non-homologous, unsubstantiated therapeutic claims are being made but are *virtually unregulated*.

A striking example is provided by certain skin- and amniotic tissue-based allograft products commercialized through the Section 361 pathway, that are marketed as wound treatments, where the validity of most of the claims being made is far from self-evident. The distributors of these products typically announce that the claims are supported by clinical data. However the studies are often underpowered, scientifically flawed and unlikely to meet FDA’s standards for valid scientific evidence. Finalizing the draft guidance on homologous use is crucial because it will clarify for industry what is and is not permissible for Section 361 HCT/Ps and will also make enforcement more straightforward.

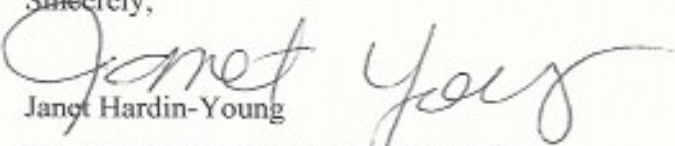
Historically, FDA has applied the concept of “intended use” in the minimal manipulation context, finding that a particular process may be “minimal” for a tissue that is intended for one use but not “minimal” when a tissue is intended for a different use. The minimal manipulation guidance has been criticized for introducing the supposedly new concept of “main function” into determinations of whether a tissue is structural or nonstructural. The reality is, that FDA has been applying this concept to minimal manipulation determinations for almost 20 years. When FDA proposed Part 1271, the Agency stated “FDA recognizes that some products may have both systemic and structural effects but *intends that a product’s primary effect be determinative*.” The term “main function” may use a new word – “main” instead of “primary” -- but the concept is well-established and, from my perspective, makes a great deal of sense. For example, in the context of wound healing, when allografts are promoted for their ability to improve the speed and quality of healing

by interacting with a wound at a cellular level, the potential impact of various processing techniques to alter the original relevant characteristics of a tissue is much greater than the impact of those same processes when the tissue is intended as a wound cover, which is merely a physical function.

In conclusion, I'd like to emphasize that wound healing products are targeted at a particularly vulnerable, chronically ill population. I'd like to urge the Agency to move as quickly as possible to finalize the guidances, retaining an approach that protects the public health and encourages innovation by providing meaningful clarity to the boundaries set forth in Section 1271.10.

We thank you for the opportunity to submit these comments and for your consideration of them.

Sincerely,

A handwritten signature in cursive script that reads "Janet Young". The signature is written in dark ink and is positioned to the right of the printed name.

Janet Hardin-Young

Co-Founder, Parenteau BioConsultants